Vandetanib: RET-positive medullary thyroid cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Vandetanib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take at the same time daily, with or without food. If a dose is missed, take it when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, disperse the whole tablet without crushing in half a glass of still drinking water, stir for about 10 minutes and drink immediately; rinse the glass with another half-glass and drink. Use no other liquid. Pharmacy must teach feeding-tube administration if needed.
- Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. electrocardiogram (ECG), potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.
- Carry the manufacturer's patient alert card and receive one with every prescription. Report fainting or palpitations urgently. Check every new medicine for heart-rhythm interactions. Diarrhoea can disturb blood salts and increase rhythm risk. Protect skin from sunlight.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____